

KING FAHD CENTRAL HOSPITAL (KFCH)

Comprehensive Stroke Program

MECHANICAL THROMBECTOMY CANDIDATE TRANSFER CHECKLIST

Document ID	SU-014-FORM-01	Version	1.0
Effective Date	_____	Review Date	_____

Patient Name / MRN	_____	Date / Time	_____
DOB / Age	_____	LKW / Onset	_____
Baseline NIHSS	_____	Weight (kg)	_____

A. Stroke & LVO Assessment

Required Check	Status	Details / Time
Stroke Code activated and treating stroke physician notified	☐ Yes ☐ No ☐ N/A	
Last Known Well / symptom onset documented	☐ Yes ☐ No ☐ N/A	
NIHSS completed and documented	☐ Yes ☐ No ☐ N/A	
Non-contrast CT completed and intracranial hemorrhage status documented	☐ Yes ☐ No ☐ N/A	
CTA/MRA or other approved vascular imaging completed when indicated	☐ Yes ☐ No ☐ N/A	
Suspected/confirmed large-vessel occlusion documented, including vessel/site	☐ Yes ☐ No ☐ N/A	
Pre-stroke functional status (e.g., mRS) documented	☐ Yes ☐ No ☐ N/A	
Mechanical thrombectomy candidacy assessed using the current approved KFCH selection protocol	☐ Yes ☐ No ☐ N/A	

B. Reperfusion Treatment Coordination

Required Check	Status	Details / Time
IV thrombolysis eligibility assessed without delaying thrombectomy workflow	☐ Yes ☐ No ☐ N/A	
IV thrombolysis administered when indicated; agent, dose and start time documented	☐ Yes ☐ No ☐ N/A	
Reason IV thrombolysis was not given documented when applicable	☐ Yes ☐ No ☐ N/A	
Endovascular / neurointerventional physician contacted	☐ Yes ☐ No ☐ N/A	
Candidate accepted for mechanical thrombectomy / angiography assessment	☐ Yes ☐ No ☐ N/A	

C. Transfer / Transport Readiness

Required Check	Status	Details / Time
Receiving service / thrombectomy team acceptance confirmed	☐ Yes ☐ No ☐ N/A	
Transfer destination / angiography suite / receiving facility confirmed	☐ Yes ☐ No ☐ N/A	
Mode of transport activated and priority communicated	☐ Yes ☐ No ☐ N/A	
Relevant CT/CTA/MRI images available to receiving team or image-transfer process confirmed	☐ Yes ☐ No ☐ N/A	
Clinical summary, medication record, laboratory results and imaging reports prepared	☐ Yes ☐ No ☐ N/A	
IV access secured and patient appropriately monitored during transport	☐ Yes ☐ No ☐ N/A	
Airway/oxygenation/hemodynamic needs assessed and stabilized for transport	☐ Yes ☐ No ☐ N/A	
Blood pressure managed according to current stroke/reperfusion protocol	☐ Yes ☐ No ☐ N/A	
NPO status maintained unless otherwise clinically directed	☐ Yes ☐ No ☐ N/A	
Family / responsible person informed of transfer plan when feasible	☐ Yes ☐ No ☐ N/A	



D. Time-Critical Documentation

Required Check	Status	Details / Time
Stroke Code activation time documented	☐ Yes ☐ No ☐ N/A	
CT start/completion time documented	☐ Yes ☐ No ☐ N/A	
CTA/vascular imaging time documented when applicable	☐ Yes ☐ No ☐ N/A	
Neurointerventional consultation / contact time documented	☐ Yes ☐ No ☐ N/A	
Acceptance decision time documented	☐ Yes ☐ No ☐ N/A	
Transport request time documented	☐ Yes ☐ No ☐ N/A	
Departure from ED/unit time documented	☐ Yes ☐ No ☐ N/A	
Arrival at thrombectomy-capable destination / angiography suite time documented	☐ Yes ☐ No ☐ N/A	
Arterial puncture (groin) time documented when available	☐ Yes ☐ No ☐ N/A	

E. Transfer Handover Record

LVO site	_____	ASPECTS / imaging summary	_____
IVT status	☐ Given ☐ Not given	IVT time	_____
BP before transfer	_____	Glucose	_____
Receiving physician/team	_____	Contact time	_____
Transport mode	_____	Departure time	_____
Destination	_____	Arrival time	_____

F. Variance / Delay / Safety Issue

Delay or variance identified	☐ No ☐ Yes — describe below
Reason / contributing factor	_____
Immediate action / escalation	_____

Sending Physician	_____	Date/Time	_____
Stroke Nurse / RN	_____	Date/Time	_____
Receiving Team / Handover To	_____	Date/Time	_____

Document linkage: Supporting form to the KFCH Stroke Unit Policy & Procedure Manual 2026, Thrombectomy Patient Selection Protocol, Mechanical Thrombectomy Selection and Post-Procedure Protocol, and Thrombectomy Workflow Chart. This checklist supports rapid, standardized transfer and does not replace clinical judgment or current approved protocols.